APEXCARE INDIA EMPLOYEE HEALTH BENEFIT PROGRAMME (AIEHBP)
CLAIMS SUBMISSION AND REQUIRED DOCUMENTS GUIDE

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Primary audience: Members, provider desks and claims intake teams

FICTIONAL TRAINING DOCUMENT - NOT AN INSURANCE CONTRACT. This document is a synthetic example created for software development, retrieval testing, and claims-workflow demonstrations. A real insurer, employer, TPA, legal team, medical team, and compliance function must review and approve all wording, benefits, timelines, forms, and regulatory references before operational use.

Document-control rule: cite the document code, section, clause, page, and version in every decision.
Interpretation rule: the signed policy schedule and endorsements override this demonstration text.

1. SUBMISSION PRINCIPLES

Documents establish identity, insured event, diagnosis, treatment, payment and amount. Missing documents must be requested specifically.

CLM-1.1 Minimum viable claim

A claim requires member and policy identification, claimant contact, provider identity, admission and discharge dates, diagnosis or presenting condition, treatment description, claimed amount and claimant declaration.

CLM-1.2 Originals and copies

Legible digital copies may start assessment. Originals may be requested for reimbursement, coordination of benefits or suspected alteration where permitted. Receipt of an original must be logged.

CLM-1.3 Alternative evidence

Where a standard document is unavailable, equivalent reliable evidence may be accepted after documented review. The system must not reject solely because a file name or format differs.

CLM-1.4 Language and legibility

Illegible, truncated, password-protected or untranslated material is PENDING_INFORMATION. It is not automatically false or fraudulent.

2. UNIVERSAL REIMBURSEMENT CHECKLIST

The following items normally apply to in-patient reimbursement.

CLM-2.1 Completed claim form
Signed or authenticated form with member, treatment, bank and declaration fields.

CLM-2.2 Member identity
Health card or member certificate plus government-issued identity where required.

CLM-2.3 Discharge summary
Final diagnosis, history, treatment, procedures, condition at discharge and follow-up advice.

CLM-2.4 Final hospital bill
Numbered bill showing provider, patient, dates, total and taxes.

CLM-2.5 Detailed bill breakup
Itemized room, professional, procedure, medicine, diagnostic, implant and other charges.

CLM-2.6 Payment evidence
Numbered receipts or acceptable digital payment proof for reimbursement claims.

CLM-2.7 Prescriptions
Signed prescriptions supporting medicines, investigations and follow-up treatment claimed.

CLM-2.8 Diagnostic reports
Reports that support diagnosis and medical necessity, not merely invoices.

CLM-2.9 Bank evidence
Cancelled cheque or verified bank details matching the authorized payee.

CLM-2.10 KYC or tax documents
Only when required by payment, identity or applicable compliance rules.

3. CONDITIONAL DOCUMENT MATRIX

Request only documents relevant to the event and reason for assessment.

CLM-3.1 Accident
Medico-legal record or police information when created, emergency notes, accident narrative, and alcohol/toxicology report only when clinically performed or materially relevant.

CLM-3.2 Implant
Implant sticker, batch or serial number, invoice and procurement record when an implant charge is claimed.

CLM-3.3 Maternity
Obstetric records, delivery notes, newborn record and birth evidence. Marriage proof is not a default medical claim requirement.

CLM-3.4 Newborn
Birth record, neonatal notes, enrollment request and relationship evidence.

CLM-3.5 Organ transplant
Authorization records, recipient and donor clinical records, donor bills and lawful transplant approval.

CLM-3.6 Dialysis or chemotherapy
Cycle plan, administration record, drug details, invoices and treating specialist advice.

CLM-3.7 Day-care surgery
Procedure note, anaesthesia record where applicable and discharge advice.

CLM-3.8 AYUSH
Facility recognition evidence, practitioner registration, case sheets, prescriptions and itemized bills.

CLM-3.9 Domiciliary treatment
Doctor certification explaining why hospital transfer was impossible or no bed was available, plus daily treatment charts.

CLM-3.10 Other insurance
Other insurer settlement letter, certified bill copies and deduction details when coordination is required.

CLM-3.11 Death claim
Death summary or certificate and lawful claimant authority where payment cannot be made to the member.

4. CASHLESS PREAUTHORIZATION DOCUMENTS

Cashless requests focus on eligibility and estimated treatment before final billing.

CLM-4.1 Preauthorization request

Network hospital request containing symptoms, provisional diagnosis, proposed procedure, estimated cost, expected length of stay and treating doctor authentication.

CLM-4.2 Supporting clinical evidence

Consultation note, investigation reports and emergency notes sufficient to assess necessity.

CLM-4.3 Final authorization evidence

At discharge the hospital submits final bill, discharge summary, procedure notes, implant details and any variance from the authorized treatment.

CLM-4.4 Authorization is not final payment

Approval is conditional on final diagnosis, actual treatment, active eligibility, policy terms and complete final records.

5. DEFICIENCY MANAGEMENT

Every deficiency request must be precise, relevant and tracked.

CLM-5.1 Single consolidated request
Where practicable, issue one consolidated list identifying each missing or unclear item and why it is required.

CLM-5.2 Response window
The demonstration workflow allows fifteen calendar days for response, with reasonable extension where justified. Production timelines must match current law and contract.

CLM-5.3 No irrelevant demand
Do not request unrelated historical records, full employment files or excessive identity documents without a recorded necessity.

CLM-5.4 Repeated request control
A previously supplied document must not be requested again unless it is unreadable, incomplete or inconsistent; the reason must be recorded.

CLM-5.5 Closure
A claim may be administratively closed after documented reminders and review, but must be reopened where policy or law requires upon receipt of evidence.

6. DOCUMENT VALIDATION CHECKS

Validation records facts; it does not invent missing values.

CLM-6.1 Cross-document identity

Compare patient name, age, sex, member ID and relationship across claim form, member record and clinical documents. Normalize harmless formatting differences.

CLM-6.2 Date chronology

Check consultation, admission, procedure, discharge, invoice and payment chronology. Impossible dates require clarification.

CLM-6.3 Amount reconciliation

Reconcile itemized bill, final bill, receipts, discounts, deposits and requested amount. Preserve currency and decimal values.

CLM-6.4 Provider validation

Verify provider name, address, registration and network status for the date of service.

CLM-6.5 Clinical consistency

Diagnosis, procedure, length of stay, prescriptions and investigations should be clinically coherent; inconsistencies require medical review.

7. FILE AND DATA STANDARDS

These standards support reliable OCR and retrieval.

CLM-7.1 Accepted files

Searchable PDF is preferred. PDF scans, JPEG and PNG may be accepted through OCR when legible. Executables and untrusted archives are prohibited.

CLM-7.2 Image quality

Target at least 300 DPI, full-page capture, correct orientation, visible edges and no glare over monetary or clinical fields.

CLM-7.3 Naming

Recommended pattern is memberID_claimID_documentType_date_sequence, without diagnosis in externally visible filenames.

CLM-7.4 Extraction confidence

Low-confidence OCR fields, handwriting, altered documents and contradictory totals require human verification against the source image.

8. CLAIM-TYPE QUICK REFERENCE

This index points to the controlling checklist.

CLM-8.1 Hospital reimbursement
Use universal checklist CLM-2 and relevant conditional items CLM-3.

CLM-8.2 Cashless hospitalization
Use CLM-4 and the procedural rules in AIEHBP-NET-004.

CLM-8.3 Pre/post hospitalization
Provide accepted hospitalization reference, dated prescriptions, invoices, receipts and reports tying expenses to the same condition.

CLM-8.4 Ambulance
Provide transport invoice showing patient, date, origin, destination and emergency purpose.

CLM-8.5 Do not infer approval
Document completeness means the file can be assessed; it does not establish coverage or payable amount.